

Medicaid Managed Long-Term Services and Supports

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Expansion of Managed Care

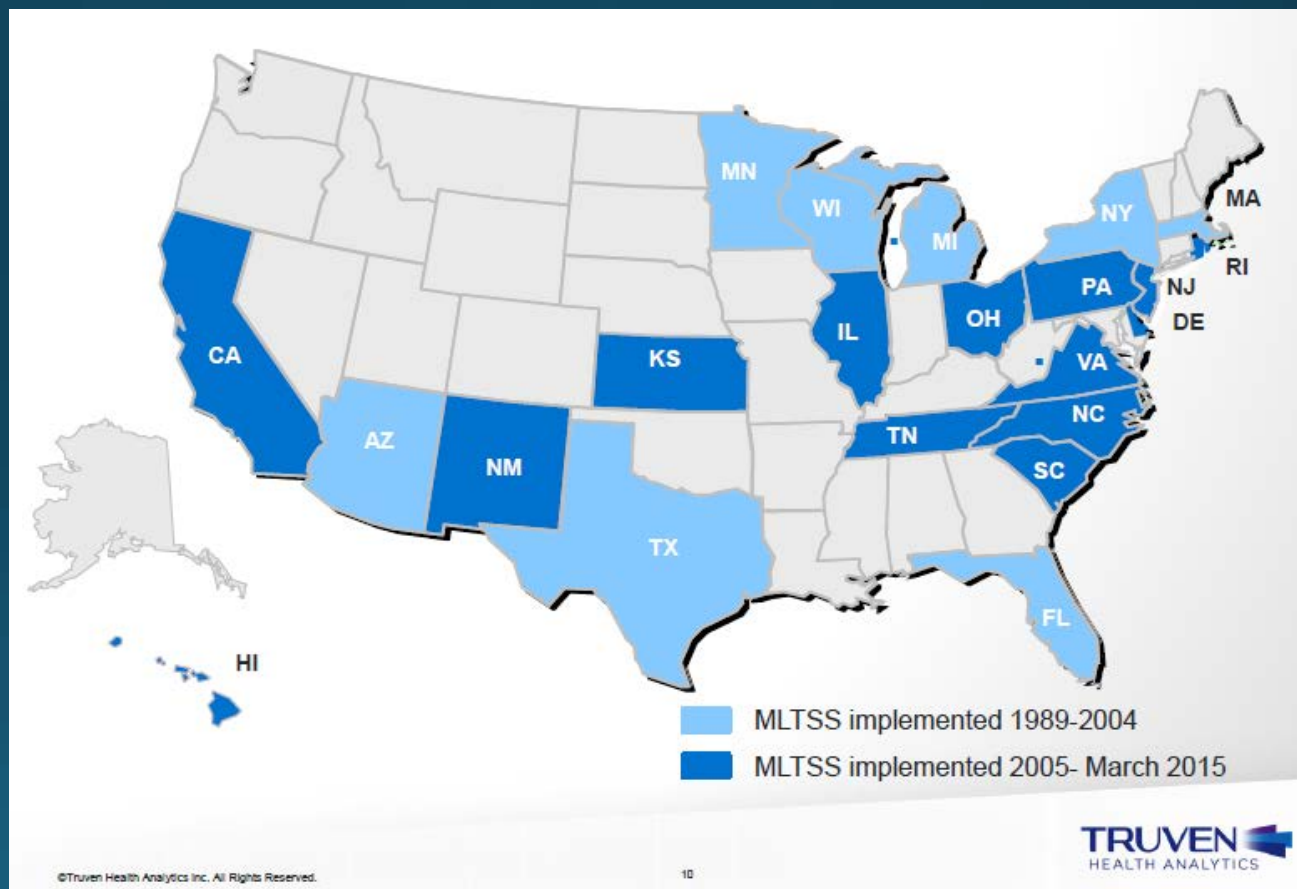
- Across the country, the vast majority (about 80%) of all Medicaid participants receive some or all of their medical, behavioral or long-term services through managed care
- All states except Alaska and Wyoming have some Medicaid Managed Care
- There are roughly 4.3 million users of long-term services and supports (LTSS) with Medicaid financing, about \$166B – mostly fee-for-service
- States have set the stage to shift much of this spending to managed care

Anticipated Growth of MLTSS

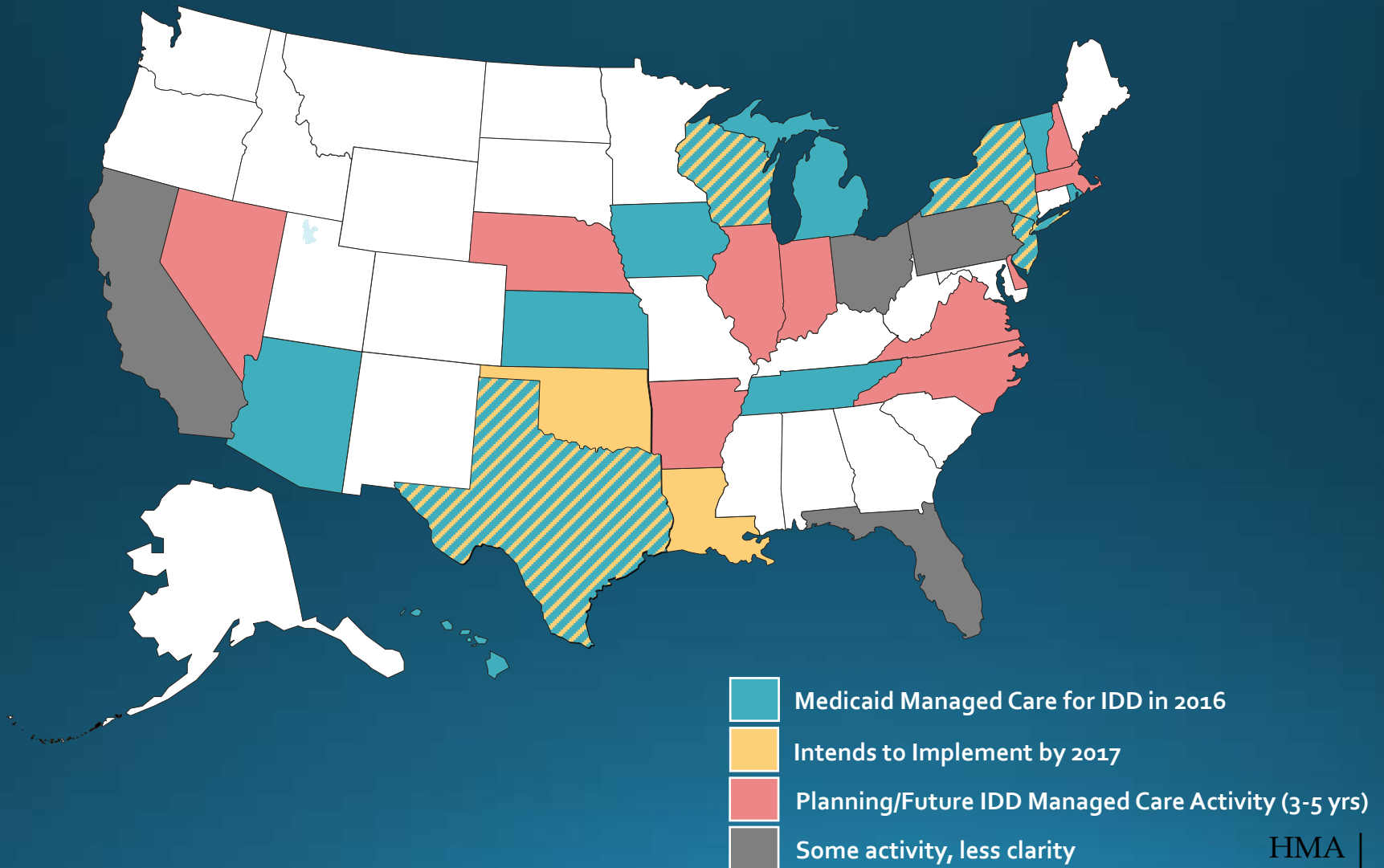
- 2014: 613,000 people accounted for \$22.7 billion.
- 2017: 1.6 million MLTSS beneficiaries are projected to account for \$56.4 billion.
- 2020: 2.7 million MLTSS beneficiaries are projected to account for more than \$95 billion.
- 2030: 4.6 million MLTSS beneficiaries projected to account for nearly \$200 billion.

Managed LTSS: A growing trend

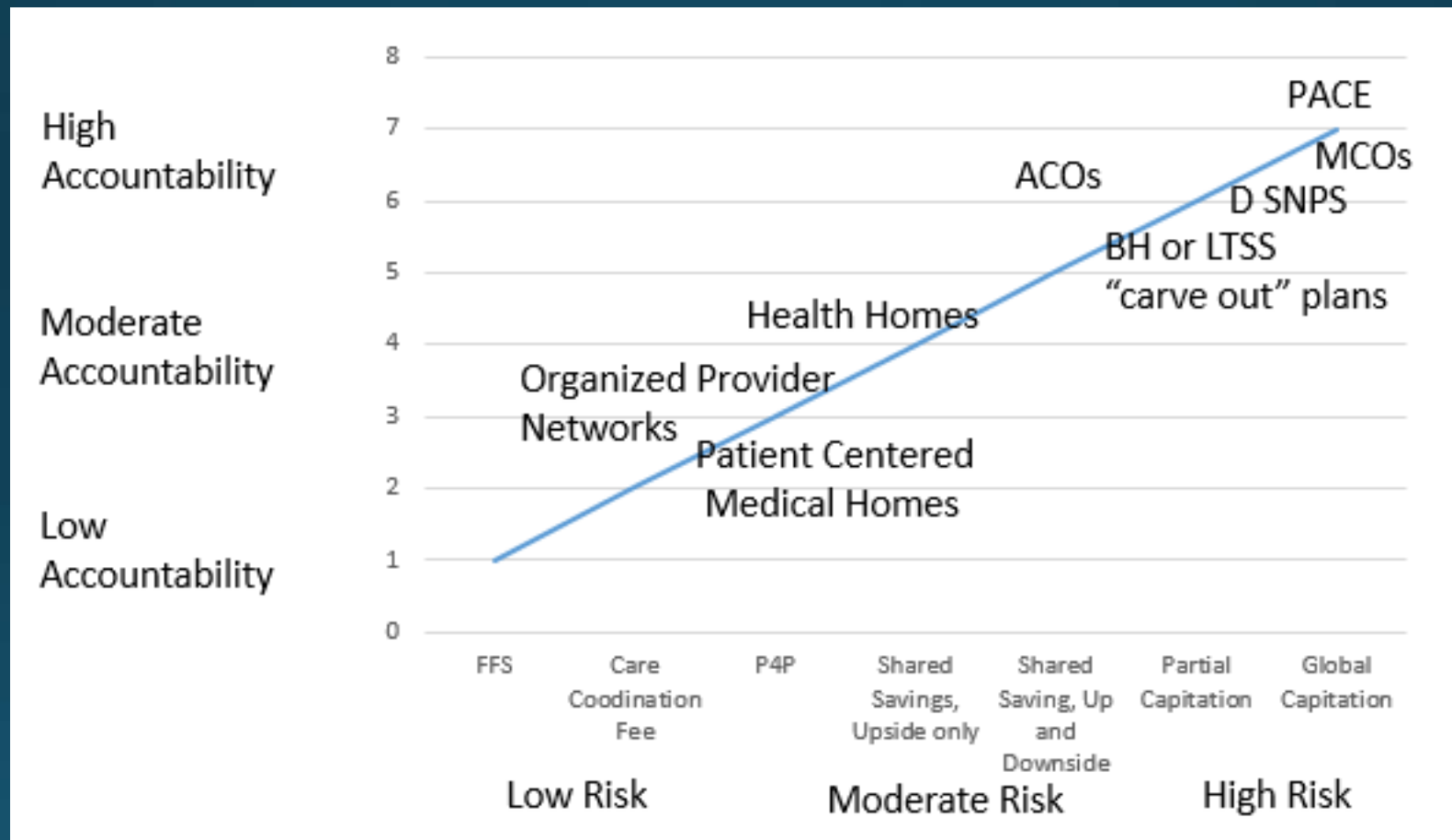
2002: 3 states
2004: 8 states
2014: 17 states
2016: 20+ states



Managed LTSS for people with IDD



Managed Risk: Many Structures



Managed LTSS: Coming to a State Near You... Why?

- Budget stability, predictable costs for states – limits financial risk for government by paying a fixed fee per beneficiary
- Integration of acute and long-term services, moving away from fragmented care
- Pay for quality, value and outcomes – not just quantity
- Accountability structure for all services
- Address waitlists, rebalancing to HCBS
- Ability to tailor unique services and supports
- Opportunities for flexibility - innovation

CMS: Ten Key MLTSS Principles

- Adequate Planning
- Stakeholder Engagement
- Enhanced Provision of Home and Community Based Services
- Alignment of Payment Structures and Goals
- Support for Beneficiaries
- Person-centered Processes
- Comprehensive, Integrated Service Package
- Qualified Providers
- Participant Protections
- Quality

<https://www.medicaid.gov/Medicaid-CHIP-Program-Information/By-Topics/Delivery-Systems/Downloads/1115-and-1915b-MLTSS-guidance.pdf>

Medicaid MLTSS: New Federal Rule

- Recognize and codify LTSS within the managed care delivery system
- Focus on both integration of care and community integration
- Shift from medical, acute care only perspective to include the provision of non-medical LTSS
- Incorporates LTSS standards into managed programs
- Requires states to include MLTSS in quality strategies
- Improves MLTSS beneficiary protections, including continuity

HCBS Quality Metrics: NQF domains

Domains for Measurement	Description of Domain
Workforce/Providers	The adequacy and appropriateness of the provider network and HCBS workforce
Consumer Voice	The level of involvement individuals who use HCBS have in the design, implementation, and evaluation of the HCBS system at all levels
Choice and Control	The level to which individuals who use HCBS are able to choose their services and control how those services are delivered
Human and Legal Rights	The level to which the human and legal rights of individuals who use HCBS are promoted and protected
System Performance	The level of accountability within the HCBS system and the extent to which it operates efficiently, ethically, and is able to achieve desired outcomes
Full Community Inclusion	The level to which HCBS integrates individuals into their communities and fosters social connectedness
Caregiver Support	The level of support (e.g., financial, emotional, technical) available for the paid and unpaid caregivers of individuals who use HCBS
Effectiveness/ Quality of Services	The level to which HCBS services are able to produce intended outcomes
Service Delivery	Aspects of services that enable a positive consumer experience (e.g., accessibility, respect, dependability, well-coordinated)
Equity	The level to which HCBS is equitably delivered and made available to a broad array of individuals who need long-term supports
Health and Well-Being	The level of integration between healthcare and other supportive services to promote holistic wellness

Managed Care: Considerations

- Family and beneficiary engagement critical as well as providers
- HCBS Settings rule: reduced reliance on facility-based LTSS
- Conflict-free choice counseling, targeted beneficiary support systems
- Olmstead/ADA expectations – residential, employment, education
- Role of “non-health” partners: education, vocational rehabilitation
- DOL regulations – homecare requirements, change in OT exemption threshold – and impact on workforce
- Opportunities to innovate, better integrate and utilize technology

Most Importantly...

- People are at the heart of a good LTSS system – regardless of financial structure
- Managed Care is a tool that can be used to benefit people – when used thoughtfully, consistent with person-centered values.
- Measuring quality and aligning financial incentives for better outcomes creates opportunities for improvement for everyone, but most importantly for the people receiving services.

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